

Clinical Documentation & Insurance Verification

Simulated Patient Case Study

This document is strictly for simulation purposes, showcasing applied skills in clinical documentation, insurance verification, and referral communication for a simulated patient encounter. Patient and clinic details are entirely fictional.

SOAP Note

Subjective

Chief Complaint: Right-sided chest pain for one week.

History of Present Illness: The patient is an adult male presenting with a one-week history of right-sided chest pain that began while lifting boxes. He describes the pain as sharp, "like a knife," with occasional radiation to the back. Severity is rated 3–4/10. Pain is worsened by lifting and deep inspiration, and relieved by ibuprofen, hot showers, and lying on his left side. He denies shortness of breath, diaphoresis, or recent trauma, and reports no prior similar episodes.

Past Medical History: Occasional heartburn; no prior similar chest pain.

Medications: Ibuprofen as needed for pain.

Allergies: No known drug allergies (NKDA).

Family History: Father died of myocardial infarction at age 60.

Social History: Denies smoking; occasional alcohol use on weekends; remote history of marijuana use.

Objective

General: No acute distress. **Cardiovascular:** Normal heart sounds, regular rate and rhythm. **Respiratory:** Lungs clear bilaterally, no abnormal breath sounds. **Musculoskeletal:** Tenderness to palpation over the right lateral chest wall and corresponding back region, with pain reproduced on palpation. **EKG:** Ordered; results pending at time of encounter. **Osteopathic findings:** Somatic dysfunction noted in the right rib region; muscle energy technique applied with mild symptomatic improvement.

Assessment

Primary diagnosis: Musculoskeletal chest pain, likely costochondritis or rib dysfunction. **Rationale:** sharp, reproducible pain worsened by movement and breathing, with normal cardiopulmonary exam, supports a musculoskeletal origin over a cardiac cause.

Differential diagnosis: Acute coronary syndrome. **Rationale:** considered given the chest pain and family history of myocardial infarction, though low severity, absence of associated symptoms, and reproducibility on palpation make this less likely. EKG ordered to rule out cardiac involvement.

Plan

Diagnostics: EKG to evaluate for cardiac abnormalities; lipid panel to assess cardiovascular risk. **Treatment:** continue ibuprofen as needed; osteopathic manipulative treatment (muscle energy technique) applied during the visit. **Patient education:** symptoms are likely musculoskeletal; avoid heavy lifting and aggravating activity; seek immediate care for worsening chest pain; multiple OMT sessions may be needed. **Follow-up:** 1–2 weeks, or sooner if symptoms worsen.

DAP Note

Data

Adult male with a one-week history of right-sided, sharp, "knife-like" chest pain radiating intermittently to the back (severity 3–4/10), onset after lifting. Worsened by deep inspiration and movement; relieved by ibuprofen, warm showers, and lying on the left side. Denies shortness of breath, diaphoresis, or trauma. Notable for occasional heartburn, no known drug allergies, and a paternal history of myocardial infarction at age 60. No smoking; occasional alcohol; remote marijuana use. Exam: no acute distress, normal cardiovascular and respiratory findings, localized right chest wall/back tenderness reproduced on

palpation, right-sided rib dysfunction on osteopathic exam. EKG ordered, results pending. Muscle energy technique performed with mild improvement.

Assessment

Primary diagnosis: Costochondritis (musculoskeletal chest pain secondary to rib/chest wall dysfunction), supported by reproducible, movement- and breath-related pain with a normal cardiopulmonary exam. Differential: acute coronary syndrome — less likely given reproducible pain and absence of systemic symptoms, but retained as a consideration given family history; EKG ordered to exclude cardiac pathology.

Plan

EKG and lipid profile ordered. Continue ibuprofen as needed; osteopathic manipulative treatment targeting rib dysfunction. Patient advised to avoid heavy lifting and aggravating activity, with education on red-flag symptoms warranting immediate care. Follow-up in 1–2 weeks or sooner if symptoms worsen.

Referral Note

To: Cardiology / Pulmonology Department

Re: Referral of Patient with Right-Sided Chest Pain

Referring a male patient with a one-week history of right-sided chest pain for specialist evaluation and further cardiac assessment. Pain began following heavy lifting, described as sharp and "knife-like" with occasional radiation to the back, rated 3–4/10, worsened by deep inspiration and movement, and relieved by ibuprofen, warm showers, and positional changes. No associated shortness of breath, diaphoresis, or trauma reported.

Past medical history notable for occasional heartburn; no known drug allergies; current medication is ibuprofen as needed. Social history negative for smoking, with occasional alcohol use. Family history significant for paternal death from myocardial infarction at age 60.

On exam, the patient was in no acute distress with unremarkable cardiovascular and respiratory findings. Localized tenderness was noted over the right lateral chest wall, reproducible on palpation, with osteopathic findings suggesting right-sided rib dysfunction. EKG was ordered; results were pending at the time of evaluation. Osteopathic manipulative treatment was performed with mild symptomatic improvement.

Working diagnosis: Costochondritis (musculoskeletal chest wall pain), with need to rule out cardiac pathology. Given the chest pain and significant family history of myocardial infarction, specialist evaluation is requested to exclude cardiac involvement and ensure appropriate risk stratification. This referral is routine to semi-urgent depending on clinical judgment and EKG findings. Current management includes ibuprofen for pain control and conservative care with activity modification.

Progress Note (2-Week Follow-Up)

The patient returns for follow-up two weeks after initial presentation of right-sided chest pain, reporting significant improvement with reduced pain intensity and frequency. Pain is now mild and intermittent, occurring primarily with heavy activity, with no new symptoms such as shortness of breath, chest tightness, or radiation.

He has been compliant with ibuprofen use and activity modification. Previously ordered investigations, including EKG, showed no evidence of cardiac abnormality. On exam, there is reduced tenderness over the right chest wall, with improving musculoskeletal findings and no new cardiopulmonary abnormalities.

Assessment: Improving costochondritis (musculoskeletal chest wall pain), responding to conservative management.

Plan: Continue conservative management with NSAIDs (ibuprofen) as needed, encourage gradual return to normal activity, and continue avoidance of heavy lifting. Patient education reinforced regarding posture and musculoskeletal strain prevention. No further cardiac workup required at this stage given improvement and negative evaluation. Follow up on an as-needed basis.

Insurance Verification & Calculation

Coverage Verification Call

Insurance Company: Aetna Provider Services — Provider Services Line

Coverage confirmed active under an Aetna PPO plan, with the treating provider in-network. No referral or prior authorization required for the relevant CPT code. Plan details: \$1,500 annual deductible (\$800 remaining), \$5,000 annual out-of-pocket maximum (\$1,200 used to date), 80/20 coinsurance once the deductible is met, and an annual limit of 20 visits (6 utilized).

Patient financial responsibility: with \$800 remaining on the deductible, the patient is responsible for the full contracted rate of the upcoming \$200 session. This payment applies toward the deductible, reducing it to \$600 for future visits. Flagged: patient will likely pay the full session rate for the next several visits given the remaining deductible balance, and has 14 of 20 annual visits remaining.

Sample Calculation

Session cost: \$200 · Specialist copay: \$40 · Annual deductible: \$1,500 (\$800 remaining) · Out-of-pocket maximum: \$5,000 (\$1,200 used) · Coinsurance: 80% insurance / 20% patient

Step 1 — Patient pays the \$40 copay at the visit; remaining bill after copay: \$200.

Step 2 — Patient still has \$800 deductible remaining, which exceeds the \$200 balance, so the entire remaining balance goes toward the deductible; insurance pays nothing because the deductible has not yet been met.

Step 3 — Total patient payment today: \$40 copay + \$200 deductible payment = \$240. Patient pays \$240; insurance pays \$0.